Nplate (romiplostim)

Original Effective Date: 5/1/2022
Revision Date(s): 5/1/2022; 10/04/2023; 4/25/2024; 11/22/2024; 10/1/2025
Review Date: 10/1/2025
Policy type: Medical Necessity
Line of Business: All GEHA Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Indication Specific Approval Criteria:

Chemotherapy-induced thrombocytopenia

- The member is 18 years of age or older; **AND**
- The member is not receiving any other thrombopoietin receptor agonist (eltrombopag, lusutrombopag etc.) or fostamatinib; **AND**
- Romiplostim is not being used to normalize platelet counts; **AND**
- Laboratory values for platelet count are within 30 days of request; **AND**
- The member is receiving active treatment with chemotherapy; **AND**
    - The member has a platelet count less than 100,000/mcL for at least 3 weeks following the last chemotherapy administration; **OR**
    - The member has had delays in chemotherapy initiation related to thrombocytopenia

Myelodysplastic Syndrome

- The member is 18 years of age or older; **AND**
- The member is not receiving any other thrombopoietin receptor agonist (eltrombopag, lusutrombopag etc.) or fostamatinib; **AND**
- Romiplostim is not being used to normalize platelet counts; **AND**
- Laboratory values for platelet count are within 30 days of request; **AND**
- The member has severe or refractory disease; **AND**
- The member has a platelet count of 50,000/mcL or lower; **AND**
- The member is diagnosed with lower risk MDS (Lower risk defined as IPSS-R (Very Low, Low, Intermediate); **AND**
- The member has failed prior therapy with one of the following: hypomethylating agents, immunosuppressive therapy, or clinical trial.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to: